

CPR Debriefing Report

Simulation Lab · 2026-08-09

Session GFI6F8 · Ventricular Fibrillation — Adult Cardiac Arrest

Team Leader: Resus Team

Guideline: AHA_2020

1. Scenario summary

Session ID	GFI6F8
Date	2026-08-09
Scenario	Ventricular Fibrillation — Adult Cardiac Arrest
Scenario type	VF
Team leader	Resus Team
Team size	6
Duration	6m 17s
Guideline	AHA_2020

Overall score	Grade	Findings	Critical	High	Moderate
100 / 100	A	0	0	0	0

2. Domain scores

Scores are derived from AHA 2020 guideline rules applied to SimMan 3G sensor data and audio transcripts. Confidence indicators reflect data completeness — scores marked medium or low confidence should not be over-interpreted.

Domain	Score	Confidence	Key finding
Cpr Quality	30/100 CI: 10–30	Low_Data confidence	—
Shock Delivery	15/100 CI: 8–15	Partial_Data confidence	—
Drug Administration	15/100 CI: 6–15	Low_Data confidence	—
Rhythm Recognition	10/100 CI: 1–10	Low_Data confidence	—
Reversible Causes (H'S & T'S)	10/100 CI: 0–10	Low_Data confidence	—
Team Leadership	10/100 CI: 0–10	Low_Data confidence	—

Team Communication	10/100 CI: 0–10	Low_Data confidence	—
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3. Protocol deviations

HIGH	R_ARREST_024	@ 0:00
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Basic airway management not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Airway
Finding ID: fnd_0001

HIGH	R_ARREST_025	@ 0:00
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Bag mask ventilation not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Breathing
Finding ID: fnd_0002

HIGH	R_ARREST_026	@ 0:00
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Defibrillator attachment not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Defibrillation
Finding ID: fnd_0003

HIGH	R_ARREST_028	@ 0:00
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IV or IO access not documented during resuscitation

Guideline reference: AHA 2025 Secondary ABCD Survey — Circulation
Finding ID: fnd_0005

MODERATE	R_ARREST_027	@ 0:00
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Advanced airway not placed during resuscitation

Guideline reference: AHA 2025 Secondary ABCD Survey — Airway
Finding ID: fnd_0004

4. Communication analysis

Detailed communication analysis requires audio transcription. Ensure lapel and ceiling mic audio files are provided for full closed-loop communication metrics.

7. Strengths

No strengths data available from narrative synthesis.

5. Reflective prompts

The following questions are mapped to specific findings detected during this session. Use them to guide the debriefing conversation with the team leader.

1. How could the team improve timing and execution for this step?
2. How could the team improve timing and execution for this step?
3. How could the team improve timing and execution for this step?
4. How could the team improve timing and execution for this step?
5. How could the team improve timing and execution for this step?

6. Recommendations

Ranked by clinical impact. Address critical and high-severity findings before moderate ones in the next training session.

1. Document airway opening technique used during resuscitation.
2. Document ventilation method used. Bag mask ventilation should begin immediately after airway opened.
3. Always document defibrillator attachment time for audit purposes.
4. IV or IO access is required for drug administration. Document route used.
5. Consider advanced airway placement for prolonged resuscitation to enable continuous compressions.